“They Look Like Us”

Drugs, addiction and family life: what children, teachers and parents told us at Neeli Mosque, Rochdale

Participants: around 50 Madrassah pupils aged 10–15, with teachers and parents (two sessions)

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INTRODUCTION

This report sets out what around 50 Madrassah pupils aged around 10 to 15, and the teachers and parents with them, told us about drugs, addiction and family life. The sessions took place at UKIM Neeli Mosque in Deeplish, Rochdale, where 73% of residents are Muslim (Census 2021).

The children described what they see on their streets, in shops and online: nitrous oxide canisters, vape shops, alcohol on supermarket shelves and dealing near places where they play. The adults spoke about stigma, denial, faith, policing and the role of the mosque. Together, their voices show how early exposure begins, how silence at home keeps problems hidden, and how much both children and adults want regular, honest teaching.

> *“So whenever I go to the shop there's this like one little area and I always want to go inside, to know what's inside, it's like drugs and everything in there.”*
>
> **— Young child, Neeli Mosque**

KEY FINDINGS

Children know more than they say. Most named alcohol, vapes, cannabis and nitrous oxide, some by street names like “balloons”.

Drugs are part of everyday places. Parks, a neighbour's door, the walk to the sweet shop, vape shops, and alcohol in Aldi and Morrisons.

Dealing happens in plain sight. A teacher: “it's always Asian people… they look like us”, often near parks where children play.

Marketing reaches children. They named “pancake flavoured” vapes and sweets that blow out smoke.

Home gives warnings, not conversations. Children expect a smack, shouting or grounding: it is “very hush hush”.

Shame delays help. Parents “can't deal with the shame”, and denial “delays the help”.

Faith can open the door. Children know drugs are forbidden in Islam. Adults spoke of mercy and the mosque as a place of support.

Children want regular teaching. Schools and mosques “hardly ever do it”. Every child asked said the community should learn more.

METHODOLOGY

Participants. Neeli Mosque's leadership asked us to run these sessions. On 10 July 2026 we held two focus groups with around 50 pupils aged around 10 to 15, during the Madrassah classes they attend five days a week. Both groups were mixed, with boys and girls seated on either side of a partition, in line with mosque practice. The second group was generally younger. Madrassah teachers, a parent and other adults also took part.

Data collection. Waffa Hameed Salik, a facilitator with lived experience of addiction from the same community, led both sessions. He used images of substances, a drug box with replica examples of street drugs, a short video and open questions on addiction, what children see, family, faith, policing and education. Both sessions were audio-recorded with the consent of mosque management and teachers.

Analysis. Mustafa Hameed, Project Coordinator, listened back to the recordings, transcribed what was said and grouped it into recurring themes. Quotes are labelled by type of speaker, and all contributions are anonymised.

Limitations. Many children were shy, and group answers mean quieter voices may be missing. We would not normally combine such different ages, but took up the request as a rare chance to hear from young people in a faith setting.

CONTINUE FOR DETAILED ANALYSIS »»

HOW THE CHILDREN UNDERSTOOD ADDICTION

We began by asking children what addiction means. Their answers came from their own lives:

> *"Imagine that you're in a candy store and you like a candy so much. You want it again and you keep having it again and again."*
> — Primary-age girl, first session

> *"Imagine you're on a diet and basically you're like really addicted to like chocolate and desserts and like you can't stop having it."*
> — Primary-age girl, first session

In the younger group, children described addiction as "something that you try and want to keep doing again" and "something that gives you a rush or enjoyment and you want to feel that again". Others said "you can never stop doing it" and simply "it's a habit".

Every answer started from pleasure and craving, not judgement. No child described people with addiction as bad. They understood it as wanting something you like and struggling to stop.

We also showed an image of someone on their phone. This was deliberate. The Salik Project UK treats addiction as a mental health issue linked to behaviour, not a moral failing that belongs only to people who use drugs. Starting with something every child knows, like a phone, shows that addiction can affect anyone. That breaks down the "them and us" thinking that feeds stigma in our communities, and makes children more likely to show compassion, speak up and seek help early, for themselves or for someone else. Asked which pictures showed something addictive, the younger group called out: "All of them!"

1. WHAT CHILDREN SEE: STREETS, SHOPS AND SCREENS

Children's knowledge was striking for their age. Most named alcohol and vaping straight away. Shown an image of nitrous oxide canisters, one boy in the younger group shouted out "Nitrous oxide!" with some enthusiasm. Some called the canisters "balloons", their street name, others "laughing gas", and some confused them with helium. In the younger group, this knowledge came from what they had seen around them and online, with a few sessions in school to boot.

There was a clear gender difference. Shown an image of someone holding a cannabis joint, the boys identified it immediately. One girl of around 10 said the person was "looking at her nails". In the first group, a teacher noted that the younger girls did not recognise it either. Asked whether they had seen nitrous oxide canisters, the girls nodded but were too shy to say more. One boy said he sees them "on the floor randomly like in a can".

Everyday places

Asked where they had seen drugs or alcohol sold or used, the younger group answered quickly:

- "Aldi sells alcohol."
- "Morrisons."
- "Cornershop."
- "Vape shop."
- "Outside club."
- "In the park I see smoking, drugs."
- "My next door neighbour's house, they smoke all the time."

Alcohol on supermarket shelves stands out. Muslim children grow up being taught that alcohol is an intoxicant to avoid, so they notice it. One young girl said she saw a man selling beer at Rochdale Interchange. Asked whether he

had seen alcohol online or in person, a young boy answered quietly: “in person.”

> *“This one time I was walking to the sweet shop, yeah, just like early, and this house just there, I smelt something weird. I smelt weed.”*
>
> — Child, first session

> *“So whenever I go to the shop there's this like one little area and I always want to go inside, to know what's inside, it's like drugs and everything in there.”*
>
> — Young child

That second child's curiosity is exactly where a trusted adult needs to step in. The facilitator told children not to approach places or people they think are involved in drugs, and to tell an adult.

Vapes and sweets

Children knew vapes were first meant to help people stop smoking. Asked what keeps young people using them, they named nicotine and “flavours”. Shown images of brightly packaged vapes, they picked out “pancake flavoured and French toast flavoured vapes”. One participant said the packs “look child friendly but they're actually not”. Yet when the “Pancake Man” design appeared, one child called out: “I love pancakes!” Children can spot the trick and still feel its pull.

Vapes are part of these children's everyday surroundings. A younger child remembered “a store that said vape store”, and in the second group one child called vape shops “drug shops”. Another had heard that “someone vapes in class”, though he wasn't sure it was true.

From our wider outreach, not these sessions: pupils we speak to in Rochdale secondary schools tell us that some children sell vapes to other children, on school grounds and after school.

> *“They were selling these sweets and they were like in a tube and it said blow and I didn't know what that meant, so I read the back and it said when you blow, smoke comes out of it.”*
>
> — Child, first session

Another child linked this to a wider worry: people “need to be aware that someone could offer you something like a sweet and it might have drugs in it.”

Online

Children named TikTok, Instagram and Snapchat as places they had seen drugs. One participant described how it feels: “That you should do it. It looks fun.” Another said content spreads between friends: “They share. It's all out there.”

Not everything online glamorises drugs. A female Madrassah teacher said what she sees on Instagram is “the aftermath”: videos showing the consequences of using substances such as nitrous oxide. She found these powerful, and they point to the kind of content that could work as prevention.

Why young people start

The younger group's answers all pointed to other people:

- “They start seeing older people using drugs so they try it and like it.”
- “They get bribed by their friends.”
- “They see it online.”
- “They see loads of people doing it so they want to try it out.”

The first group added peer pressure, flavour and smell, and thinking it is fun. Older young people, friends and social media are the route in, so role models matter.

One participant gave a different kind of answer: young people might start because they are “not happy with themselves in their life”. It was the only answer that linked drug use to feelings rather than to other people or to curiosity. That matters. Adults in recovery often describe using substances to cope with pain, stress or low self-

worth. If children only learn about peer pressure, they may miss the warning signs in themselves or a friend who is struggling inside. Prevention needs to help children talk about how they feel, not only about what they see.

Nitrous oxide

Both groups watched a short video of a young South Asian Muslim man talking about his friend, who died after using nitrous oxide. His friend had believed taking vitamin B12 would protect his nerves. The facilitator explained that this is a myth: many people end up in wheelchairs or losing feeling, and in this case someone died.

The video landed. In the first group, a participant retold it in their own words: "He was talking about how his friend died from a balloon." In the younger group, the first piece of advice children reached for afterwards was: "tell them to stop taking drugs because they might die."

A female Madrassah teacher drew out a deeper lesson. The young man in the video had lost his friend, she said, and might feel guilty about what more he could have done. That is why it matters to tell friends not to take drugs, and never to join in or encourage it. Seeing the story through the eyes of the friend left behind, rather than only the person who died, helped children see their own part in looking out for each other.

Nitrous oxide needs particular attention in South Asian communities. It has become part of youth culture for some young British South Asian men, and its harms are now being called out from within. In recent months, TikTok creators with huge followings, including Farah Shamsi and Gorcal, have used their platforms to warn against it.

The harm is real. A 2023 study of 119 young people treated for nerve and spinal cord damage from nitrous oxide in London, Birmingham and Manchester found that 57% were Asian or Asian British. Three in four were male, and their median age was 22. We also hear of young men facing criminal charges for driving under its influence.

The Salik Project UK believes the problem is vastly underreported. Within the community, nitrous oxide is still often seen as harmless fun, and even as "less haram" (less forbidden) than drugs such as cannabis. The children's easy familiarity with "balloons" suggests that view is already reaching them.

2. "THEY LOOK LIKE US": DEALING IN PLAIN SIGHT

Some of the strongest observations came from the adults. An older female Madrassah teacher described what she regularly sees near the local cash and carry: young Asian men stopping their cars, walking across the road "like they own it", and exchanging drugs. "It's so common around here," she said, and you can only hope something gets done about it.

Another female teacher said the words that gave this report its title:

> *"But the thing is, it's always Asian people and Muslims, so you think that you know it's okay cuz they look like us... but around the parks where kids are playing, street corners, it's actually quite sad to see that."*
>
> — Female Madrassah teacher

Children see it too. One boy in the younger group said he regularly sees people selling drugs from cars where he lives.

The teacher's words hold two feelings at once. Familiar faces can make dealing feel normal, even "okay". Yet she called it sad, especially near places where children play. When danger looks like your own community, it is harder for children to recognise it as danger.

Her observation points to something British South Asian communities recognise. The young local dealer, "on the graft" (street slang for dealing) and known as a "shotta" (a dealer), has become a familiar character, and comic sketches of him are common on TikTok. This is not a picture of South Asian young people in Rochdale, most of whom are nothing like it. But it is a trope rooted in a reality the community has to face.

From our wider work, not these sessions: in The Salik Project UK's conversations with young people who have been, or still are, involved in dealing, three drivers come up. They are deprivation and few other routes to earning; a "gangster life" made to look like success through music, social media and street status; and exploitation by older dealers who recruit young people to sell for them.

For a child, this is the backdrop. When the dealer is a familiar type, joked about online and sometimes a face from the neighbourhood, he is harder to see as a danger and easier to see as someone to become.

3. PARENTS, SILENCE AND FAMILY REALITIES

When the first group was asked whether drugs or addiction were ever discussed at home, the room fell silent. A female teacher broke the silence: these conversations simply are not happening at home, she said, and that is a real shame.

In the younger group, some children did hear about drugs at home, but only as a warning:

> *"They always say don't use it, don't use it, all the time."*
>
> — Child, second session

> *"My mum always tells me not to do drugs."*
>
> — Child, second session

"All the time" and "always" suggest a message repeated often but never explained. Children are told not to use drugs, but not what drugs do, why people use them, or what to do if a friend is in trouble.

Asked how South Asian families deal with someone taking drugs, children in the first group answered one after another:

- "Very hush hush."
- "They give you a smack."
- "They shout at you."
- "Ground you."

The younger group found the question confusing and answered with advice instead. That confusion is telling in itself: the idea of a family openly dealing with drug use was outside their frame of reference.

Children also described a community where everyone knows but no one speaks. Asked whether they had heard of anyone using drugs, one participant said: "You hear about it, everyone knows the person and what they do but no one really talks about it, like." In the younger group, the stories came second-hand, through friends: "my mate, from my mate".

Put together, children hear three things at home: silence, warnings and the threat of punishment. None of these invites a question. So children fill the gap with what they learn from friends, the street and their phones. When a female teacher asked the children, "Would you keep it to yourself?", she was naming exactly this risk. The facilitator closed by urging them to speak to their parents or someone older if they, or someone they knew, were struggling.

4. SHAME, IZZAT AND SUFFERING IN SILENCE

Izzat is family honour and standing in the community. *Sharam* is shame, especially shame brought on a family in front of others. It has become almost a cliché to say that izzat shapes how South Asian families respond to addiction. But it has come up in every focus group The Salik Project UK has run so far, because it is central to the South Asian experience. These sessions were no different.

A parent explained why families stay quiet:

> *"In our culture there's a stigma around it, that's why people don't want to discuss it, otherwise everyone will know that family has a problem around it."*
>
> — Female parent

Her words show that the shame is collective. A problem with one person becomes a problem for the whole family's name. An older female student described what that leads to:

> *"Sometimes parents will be in denial and instead of facing it and dealing with it, they will actually cover it up because they can't deal with the shame."*
>
> — Older female student

A female teacher spelled out the cost. Denial "delays the help", she said, and soon the "child is or the person is deep into the drugs or the alcohol and then it gets harder for them to recover." A teacher who is also a parent admitted the same instinct in herself and others: "as parents or sisters or brothers we tend to turn away, don't we? We don't want to know the problem."

The teacher with 15 years' experience spoke from what she sees working with families across Deeplish:

> *"I see it around this community and I know how it affects people's families. It doesn't just affect the addict, it affects their home, you know, people's families break up, their whole lives, everything. So... I think more needs to be done."*
>
> — Female Madrassah teacher, 15 years' experience

Across these voices, shame works at three levels. Families hide problems from the community. Parents hide them from themselves through denial. And, as Section 6 shows, people hide what they know from the authorities for fear of being found out. Each layer delays help.

It is significant that an older student named this so clearly. Young people can see the pattern their parents are caught in. Challenging it does not mean attacking culture or faith. It means a different understanding of honour, in which seeking help early protects the family rather than shaming it.

5. FAITH, THE MOSQUE AND RECOVERY

For the children and adults in these sessions, Islam was not a separate topic. It was the lens through which they made sense of drugs, addiction and recovery. Every question about help drew answers rooted in faith. For public health, understanding this is key: for many Muslim families, faith is a way of life, not a private add-on.

Children went first to Islam's clear rules. "They know it's haram so they might stop," said one, using the word for what is forbidden in Islam. Another said: "The prophet would say that it damages and shortens your life." A female Madrassah teacher added:

> *"The way I see it is in Islam harming yourself whether through drugs or alcohol is haram. That's the first thing you come across in Islam isn't it, that your body should be looked after by you because of what Allah has given to you."*
>
> — Female Madrassah teacher

Asked what the Prophet Muhammad (pbuh, "peace be upon him") would say to someone struggling, one participant said to "seek out a professional for guidance and ask him to do du'a for us" (a personal prayer). In one sentence, that answer brings together professional help and faith, rather than choosing between them. A teacher who is also a parent reflected:

> *"I think that question is such a strong statement because as parents or sisters or brothers we tend to turn away, don't we? We don't want to know the problem. But when you put it into that question, what would the Prophet (pbuh) say, he would never turn his back on his ummah. So it makes you think."*
>
> — Female teacher and parent

The *ummah* is the worldwide Muslim community. Framing the question through the Prophet's example turned a conversation about shame into one about compassion. Another woman spoke about both mercy and punishment:

> *"Islam is the answer to everything for us as Muslims, and a lot of people don't read the Quran in English. What would the Prophet (pbuh) say? He would say don't despair, that Allah forgives all sins... and if you ask Allah to help you, Allah will come running to you."*
>
> — Female adult attendee

She went on to say that in Islam "the punishment is so severe that Allah would end that corruption". Her answer was honest. It held both sides of how many Muslims understand their faith: mercy for anyone who turns back, and firm consequences as a deterrent. Her remark that many people "don't read the Quran in English" also matters. Many families know the rules but not the teachings on mercy and hope behind them.

A female student spoke about community:

> *"I think in Islam we underestimate community and how big it is and how we're supposed to be there to support each other. So if somebody is struggling, they're supposed to have a support system... try get away from those people, to make contact with the masjid, make friends with people here, people who are able to help you out, and that's what a masjid is supposed to do, build a community."*
>
> — Female student

"This is not the place"

Not everyone agrees the mosque should talk about drugs. A female Madrassah teacher reminded the facilitator of a conversation they had had a week earlier, after an awareness stall was set up at the mosque:

> *"A few individuals came up and said, 'Bro, this is not the place to be holding a stall.' But where is the place? This is where people are coming, repenting..."*
>
> — Female Madrassah teacher

Her question goes to the heart of this report. If people come to the mosque to repent and seek help, the mosque is exactly the place. A woman who has lived in Deeplish all her life agreed, and told the children where she would send a friend in trouble:

> *"Go to the masjid and speak to that brother, because growing up in Deeplish all my life before there wasn't anything like this... it should be that people can go to the masjid for support... but masjid need to work with other partners to help that person, but it has to be the masjid because this is the place where Allah's mercy is."*
>
> — Female adult attendee

The female Madrassah teacher also suggested ways to reach more people: using Friday sermons to spread the message, and producing literature grounded in the Quran and Sunnah (the teachings and practice of the Prophet Muhammad), "because those people that come to you, when they see that there's a quote from the Quran or from the sunnah, they won't sleep." A message carries far more weight when it comes with the authority of faith.

6. POLICING, REPORTING AND FEAR

We asked whether the police are doing enough to tackle drugs in Rochdale. The answers were among the most heartfelt of both sessions.

In the first group, views were mixed. One participant said: "Yeah, I think they're doing a good job." A younger boy disagreed:

> *"When there's trouble going in the street with boys doing drugs, there's never any police there."*
>
> — Younger boy, first session

An older boy said simply "no". Asked why, he said: "because there's still so much drugs present."

The younger group all said no. One child said he regularly sees people selling drugs from cars where he lives, and people vaping. Another pointed to the number of "drug shops", meaning vape shops, on Rochdale's high streets. Children judge the police by what they see. While dealing stays visible on their streets, they conclude that not enough is being done.

Asked whether community leaders were doing enough, one child said "I think they need to do a bit more." Another went further:

> *"I don't think they are doing enough. If you look at what actually be sold and what can be bought, I think they need to cut down on what can be sold, then maybe addiction won't go up."*
>
> — Child, second session

Community leadership

The children's sense that community leaders "need to do a bit more" matches what we hear across our work. In the two years The Salik Project UK has been operating, senior figures from within South Asian communities, including local councillors, have rarely mentioned drugs or addiction in public, and the issue does not come up at election time. When the Advisory Council on the Misuse of Drugs published its report on drug use in ethnic minority groups, we asked Rochdale's MP, Paul Waugh, for his response. We have not yet received one.

This is not about blaming individuals. Leaders are part of the same community and face the same stigma as the families we hear from. But the silence children describe at home is mirrored at the top. Until leaders are willing to name the problem, families have little reason to believe it is safe to speak.

Why dealing is hard to stop

The adults explained why street dealing is so hard to stop. A female Madrassah teacher drew a distinction between organised gangs and street-level dealers:

> *"I think that if it's an organised crime, like it's a gang and they've been telling them for a year or two, then they'll get somewhere. But I think these people who are dealing drugs are too quick, so they turn up on your street, handover, take and they're gone. By the time you've called the police or shouted to somebody, they've gone."*
>
> — Female Madrassah teacher

She mentioned a documentary about a Rochdale gang that police investigated for years before finally breaking it up. Her point was that the police can tackle organised crime over time, but the quick handovers children see every day slip through.

The facilitator stressed that reporting matters, because a lot of inaction comes down to under-reporting from within South Asian communities. One woman asked the room: "Do you think people live in fear?" The answer was yes. Women in our earlier focus groups told us the same: many fear being found out if they report. Where the people dealing are known faces in the neighbourhood, reporting them is a real risk.

Others wanted tougher consequences:

> *"I think they are trying their best but I don't think the punishment is strict enough, and I think Allah has given punishments to deter people from doing things again, and they need to be a lot stricter with the prison sentences because honestly, brother, prison sentences are a joke."*
>
> — Female adult attendee

Another woman made the same point: people "go to jail and have the best time of their lives after a year and come back out and then end up back in prison."

Together, these voices describe a gap in trust. Children see dealing go unchallenged. Adults see quick handovers the police cannot catch, sentences that do not deter, and neighbours too afraid to report. For public health and the police, this means official figures are likely to understate what is happening in areas like Deeplish, and that safe, anonymous ways to report are essential.

7. WHERE SCHOOLS AND MOSQUES ARE FALLING SHORT

Children were clear that they want more teaching, and more often:

> *"I think mosques need to discuss it a bit more and in schools because they hardly ever do it."*
>
> — Child, second session

> *"They need to do it more in like assemblies and how often they do it."*
>
> — Child, second session

Another said simply: "I don't think schools discuss it enough." Asked whether the community should learn more about drugs and addiction, every child who answered said yes:

- "Yes, because they won't get damaged or won't get hurt."

- "Yes, because it can protect people from getting addicted."
- "Yes, because they should know how bad drugs are and how they can get addicted to it and they can't stop."
- "Because people might not know the bad side effects of drugs."
- "Yeah, because they need to be aware that someone could offer you something like a sweet and it might have drugs in it."

These answers are about protection. Children are not asking to be frightened. They want to know enough to keep themselves and others safe. And they are asking about frequency as much as content: "how often they do it" matters to them.

The adults agreed. The teacher with 15 years' experience said:

> *"I have been a madrassah teacher now for 15 years and we've never had anything like this. I'm not sure if other teachers bring it up in their class but I make sure I do."*
> — Female Madrassah teacher, 15 years' experience

That nothing like these sessions had happened in 15 years is extraordinary. It shows how much depends on individual teachers choosing to raise the subject. Children attend Madrassah five days a week. That is far more regular contact than any visiting service has, and at the moment it is almost entirely unused for this purpose.

8. CREATIVE ENGAGEMENT AND HELPING A FRIEND

The sessions used images, a short video, a drug box with replica examples of street drugs, and Salik literature, which children explored at the end. They were also built around lived experience.

The facilitator asked children to guess which substance he had been addicted to. In the first group, one person guessed "beer". Then one child, with complete confidence, suggested "Dr Pepper", and the room burst out laughing. Some in the second group already knew his story and guessed alcohol.

He told them how his addiction moved from cigarettes to cannabis, other substances and alcohol, and how long it took him to come to terms with it. Some children were visibly shocked. Hearing this from someone who looks like them, from their own community, made the subject real in a way a leaflet cannot.

Lived experience alone is not enough. What made these sessions work was a facilitator from the community itself, who understands its cultural and religious vernacular: the weight of izzat, the language of haram and mercy, and the family pressures children face. He could talk about addiction in terms the children and adults recognised, without anything being lost in translation. That is why The Salik Project UK is now engaging more people from within the community to lead this work.

Asked what they would say to a friend struggling with addiction, children answered:

- "Just remind them that they are not alone and you can get out of this. It's just something that needs work."
- "Pray to God."
- "I would say stop going to the area where those bad things happen."
- "Tell a trusted adult." (an older boy)
- "Tell them to stop and ring those that can help them with addiction."
- "Explain why it's bad for them."
- "Tell them about the bad side effects."
- "Tell them to stop doing it because they can't function when they are doing it."

One younger student suggested "drink water", which set the room laughing again. That ease mattered. Children were relaxed enough to joke while talking about something serious, and humour helped the shyer ones join in.

These answers show children who see themselves as helpers. Most picture help as relationships: friends, trusted adults and God. One child named professional help directly, and one first answer, “it's just something that needs work”, shows an understanding of recovery as a process rather than a single decision.

Conclusion: What We Learned

Children aged around 10 to 15 at Neeli Mosque already know a great deal about drugs. They smell cannabis on the way to the sweet shop, see alcohol on supermarket shelves, recognise “pancake flavoured” vapes, and know nitrous oxide as “balloons”. Many were too shy to say all they knew, but what they did say shows exposure starting at primary-school age.

Their teachers see the same world from the other side: dealing near the cash and carry and in parks by people who “look like us”, and families broken by addiction. At home, children hear silence, warnings and the threat of punishment rather than conversation. Stigma, shame and denial keep families quiet, and fear keeps neighbours from reporting what they see.

Faith ran through every answer. Children knew drugs are haram. Adults spoke of mercy, community, and the mosque as “the place where Allah's mercy is”. Views on the police and punishment were mixed and honest.

The clearest lesson is about the mosque. Mosques are the heartbeat of communities like Deeplish, sometimes far more than community centres, and many already act as community centres. Children attend Madrassah five days a week. Families come to the mosque for prayer, advice and support. This is a strength that services should actively build on, by empowering mosques rather than simply visiting them.

Until now, engagement between services and mosques has been poor, intermittent and sparse. Too often it has meant one-off visits from people with little or no cultural or religious knowledge. That has to change, and it will not happen for free. Mosques run on a shoestring. Running regular sessions will need funding. It will also need trained people: either imams with proper support, or paid community connectors from within the community, rather than simply adding to imams' existing workload.

Participants are telling us that the problem is bigger and more hidden than anyone has acknowledged. As awareness grows, more people will come forward for help, including families who are quietly supporting someone with addiction on their own. When they do, public health services must be ready with support that understands their culture and faith. Small charities and CICs like ours can open the door, but we cannot be expected to carry this work alone. It will also take community leaders willing to break the silence.

Most of all, it needs a lasting relationship between local recovery services and the mosques. The children and adults at Neeli Mosque asked for more teaching, more often. A teacher of 15 years had never seen a session like this. With the right investment and partnership, the mosque could become a regular, trusted place for honest, faith-grounded prevention.

RECOMMENDATIONS

The priorities below come from what the children, teachers and parents told us. Where a specific delivery model is proposed, such as the Madrassah pilot, it is The Salik Project UK's practical recommendation for turning those findings into action.

1. Keep drug education up to date

Teaching should cover what children actually see: nitrous oxide, flavoured vapes, vapes sold between pupils, novelty sweets, alcohol on sale locally, and myths such as B12 protecting against nitrous oxide. It should take the appeal of drugs seriously, help children talk about how they feel, and be reviewed at least once a year.

2. Use creative methods and culturally matched lived experience

Images, short videos, replica items, humour and a facilitator with lived experience who comes from the children's own community, and understands its cultural and religious vernacular, kept them engaged and opened up honest answers. Children already see themselves as helpers, so young peer leaders should be trained to carry the message on.

3. Help parents begin conversations earlier

Parents need simple words for starting the conversation and help with their first reaction, so a child who speaks up is met with calm rather than a smack or shouting. Short guides should build on our work with South Asian women and be available in English, Urdu and Bengali.

4. Fund mosques to deliver this work

Mosques run on a shoestring. Regular sessions need proper funding, and paid community connectors or properly supported imams to deliver them, rather than adding to imams' existing workload. Friday sermons and literature grounded in the Quran and Sunnah can carry the message further, as one teacher suggested.

5. Pilot a monthly Madrassah module

A short monthly session on drugs, vapes, online pressure and wellbeing should be piloted in one or two mosques, with separate sessions for primary and secondary ages. Simple anonymous tablet polls would let shy children answer and track change over time.

6. Support existing youth groups and grassroots networks

Work with the youth groups, mosques and community organisations that families already trust across Rochdale, rather than starting from scratch. Share materials and plan sessions together.

7. Build lasting partnerships between recovery services and mosques

As one attendee put it, the “masjid need to work with other partners”. Commissioned services should agree a named contact and a clear referral route with each mosque, and send staff with cultural and religious understanding, so engagement is regular rather than one-off.

8. Act on shops and make reporting feel safe

Trading Standards, Public Health and community partners should check local shops for vapes and novelty sweets that appeal to children, and work with schools on vapes sold between pupils. Police and partners should promote anonymous ways to report dealing, recognising that many people in the community fear being found out.

KEY TERMS

Madrassah	A mosque school where children learn the Quran and Islamic studies outside normal school hours
Masjid	Mosque
Imam	The religious leader of a mosque
Izzat	Family honour and standing in the community
Sharam	Shame, especially shame brought on a family in front of others
Haram	Forbidden in Islam

Ummah	The worldwide Muslim community
Sunnah	The teachings and practice of the Prophet Muhammad
Du'a	A personal prayer or supplication
pbuh	“Peace be upon him”, said by Muslims after the Prophet Muhammad's name
“The graft”, “shotta”	Street slang for drug dealing and for a drug dealer

Sources: Census 2021, Deeplish MSOA (E02001146), religion — censusdata.uk · Queen Mary University of London (2023), “Young people with nerve damage from nitrous oxide use are most likely to be Asian men” — qmul.ac.uk.

About Us & Our Collaborators

ABOUT THE SALIK PROJECT UK (ROCHDALE)

The Salik Project UK is a grassroots Community Interest Company working around two connected aims: breaking the stigma surrounding addiction in South Asian communities, and helping recovery services become more culturally informed and responsive to the people they serve. We do this through community education, outreach, research, lived experience and partnership working.

We are not a treatment provider. We work with recovery services, Public Health and other partners to improve access to support and make sure South Asian voices shape the services intended for them.

About UKIM Neeli Mosque & Islamic Centre

UKIM Neeli Mosque & Islamic Centre was established in Rochdale in 1969. For more than 50 years it has been a centre of worship, learning and community life in Deeplish. Through education, culture and charity work, it aims to create an inclusive space for the whole community, and its Madrassah teaches children from across the local area.

The mosque's leadership asked for these sessions and opened its doors, teachers and pupils to make them happen.

About High Level Northern Trust

High Level Northern Trust is a Rochdale-based charity and recovery service supporting people affected by addiction. It provides a safe, welcoming space for people in recovery, while also offering guidance and support to their families and wider support networks.

About Action Together

Action Together supports voluntary, community, faith and social enterprise organisations across Rochdale, Oldham and Tameside through funding, advice, training and partnership building. It has provided funding to The Salik Project UK, helping us deliver community-based addiction awareness, education and engagement work in Rochdale.

With Special Thanks to
Neeli Mosque, Rochdale